

Endocrinology Variant

- 1.* Regarding the mass effects of pituitary adenomas.
 - a. Compression of the optic chiasm produces binasal hemianopsia
 - b. The headache correlates with tumor size
 - c. Compression of the frontal lobes produces personality disorders
 - d. Compression of the temporal lobe produces anosmia
 - e. Compression of the third cranial nerve produces facial numbness
- 2.* Regarding thyroid hormones.
 - a. They stimulate the normal development of the central nervous system
 - b. They decrease gut motility
 - c. They decrease adrenergic sensitivity
 - d. They decrease cardiac inotropism and chronotropisms
 - e. They decrease the production of erythropoietin
- 3.* AVP:
 - a. Increases water reabsorption at the level of the proximal convoluted tubule
 - b. Decreases vascular resistance
 - c. Is stimulated by a decrease in plasma osmolality
 - d. Is the main hormone that controls the water balance in the body
 - e. Acts on V1R receptors in the kidney
- 4.* Lab values in primary adrenal insufficiency:
 - a. Neutrophilia
 - b. Lymphopenia
 - c. Polycythemia
 - d. Hypoglycemia
 - e. Hypokalemia
- 5.* Regarding levothyroxine use in pregnancy:
 - a. Any TSH>2.5mUI/L needs treatment
 - b. The target TSH in the first trimester is 3 mUI/L
 - c. TSH, FT4 and FT3 must be evaluated every trimester
 - d. Breastfeeding is not allowed
 - e. Supplemental therapy of the hypothyroid mother will also sustain the fetus
- 6.* T3 replacement therapy:
 - a. Is recommended in the elderly
 - b. Is recommended in patients with cardiac pathology
 - c. Is more active than T4 replacement therapy
 - d. Is recommended in the first trimester of pregnancy
 - e. Is recommended in the third trimester of pregnancy
- 7.* Hyperprolactinemia can be encountered:
 - a. During administration of apomorphine
 - b. During treatment with pergolide
 - c. In hyperthyroidism
 - d. During hypoglycemia
 - e. In pseudohypoparathyroidism
- 8.* Regarding MEN1 syndrome:
 - a. Has an incidence of 1:3000
 - b. Menin is a protooncogene

- c. It is caused by activating mutations in *merit*
 - d. Comprises parathyroid hyperplasia in 95% of
 - e. Comprises medullary thyroid cancer
- 9.* Pheochromocytoma comprises:
- a. HTA in 50% of cases
 - b. Headache in 40% of cases
 - c. Diaphoresis in 30% of cases
 - d. Sweating in 15% of cases
 - e. Palpitations in 60% of cases
- 10.* Aldosterone:
- a. 90% is bound to albumin and CBG
 - b. 10% circulates free
 - c. Has a half-life of one day
 - d. Stimulates renal tubular potassium reabsorption
 - e. Induces expression of collagen genes
11. Regarding hypothalamic neuropeptides:
- a. CRH stimulates the secretion of TSH from thyrotrope cells
 - b. TRH is secreted by the paraventricular nucleus of the hypothalamus
 - c. TRH stimulates the secretion of prolactin from lactotroph cells
 - d. Dopamine stimulates the secretion of gonadotropins from gonadotroph cells
 - e. Somatostatin inhibits GH secretion from somatotroph cells
12. Regarding hematoxylin eosin stain of the anterior pituitary cells:
- a. Somatotroph cells are acidophilic
 - b. Lactotroph cells are basophilic
 - c. Corticotroph cells are acidophilic
 - d. Thyrotroph cells are basophilic
 - e. Gonadotroph cells are basophilic
13. Nonfunctioning pituitary adenomas:
- a. Comprise 60% of anterior pituitary adenomas
 - b. Do not cause any symptoms
 - c. Are not associated with clinical evidence of hormonal hypersecretion
 - d. Can cause hyperprolactinemia due to pituitary stalk compression
 - e. Can cause hypopituitarism
14. Indications for craniotomy as a treatment modality in pituitary adenomas:
- a. Small pituitary adenomas (< 2cm)
 - b. Visual tract compression
 - c. Invasion of frontal fossa
 - d. Extensive posterior invasion
 - e. Solid tumors (fibrous consistency)
15. Regarding treatment of prolactinomas:
- a. First line treatment is surgery
 - b. Bromocriptine has a long half-life
 - c. Cabergoline has poor tolerance
 - d. Cabergoline is administered weekly
 - e. Cabergoline decreases tumoral volume in most cases
16. Direct effects of growth hormone
- a. Protein synthesis

- b. Gluconeogenesis
 - c. Myelin synthesis
 - d. Cartilage growth
 - e. Neuronal survival
17. Regarding first generation somatostatin analogues(octreotide, lanreotide)
- a. Bind to somatostatin receptor subtype SSTR2 SSTR5
 - b. The long acting formulations are administered
 - c. GH normalization occurs in 30% of cases
 - d. They induce a 20% decrease in tumor size
 - e. Adverse effects include: gallbladder lithiasis: hyperglycaemia, nausea, vomiting, constipation
18. Regarding hypopituitarism:
- a. TSH deficiency induces thyroid failure, more severe than the peripheral disease
 - b. TSH deficiency is accompanied by goiter
 - c. Gonadotropin deficiency induces low libido and erectile dysfunction in men
 - d. ACTH deficiency causes weakness, anorexia, weight loss and vomiting
 - e. Prolactin deficiency causes absence of lactation
19. Regarding glucocorticoid replacement therapy in hypopituitarism:
- a. Hydrocortisone is administered in doses of 5 – 7.5 mg/day
 - b. Prednisone is administered in doses of 15-25 mg/day
 - c. It is associated with mineralocorticoid replacement therapy
 - d. During stress conditions, the dose is increased 3-10 times
 - e. During follow up, hormonal blood assays are of no use
20. Oxytocin
- a. It is a protein comprising 191 amino acids
 - b. It is produced as a response to suckling, parturition and orgasm
 - c. Has a well defined role in men
 - d. Oxytocin receptors are augmented during pregnancy
 - e. Oxytocin can be used therapeutically to induce labor
21. Concerning adenomas as a treatment modality for pituitary adenomas
- a. Stereotactic irradiation in a single or five fractions of 10 Gy
 - b. The response of functional pituitary adenoma to stereotactic radiosurgery occurs in 5-10 years
 - c. Fractionated conventional radiotherapy involves 25-30 daily sessions, each of 1-1.5 Gy
 - d. Hypopituitarism only occurs after fractionated conventional radiotherapy
 - e. Acute adverse effects of radiotherapy include alopecia, weakness and optic nerve edema
22. Regarding the endocrine syndrome in prolactinomas:
- a. Galactorrhoea is the most frequent symptom in women
 - b. Involves vaginal dryness and dyspareunia in women
 - c. Involves decreased arousal and erectile dysfunction in men
 - d. The diagnosis of prolactinoma is established early in men
 - e. Bone demineralization only occurs in women
23. Treatment of nephrogenic diabetes insipidus:
- a. Thiazide diuretics in combination with a low sodium diet can diminish the degree of polyuria

- b. A loop diuretic lowers the urine output
 - c. Amiloride can reduce polyuria
 - d. Indomethacin enhances AVP response
 - e. Lithium can be used as therapy
24. Decreased TBG levels can be encountered:
- a. During treatment with androgens
 - b. During treatment with 5-fluorouracil
 - c. Nephrotic syndrome
 - d. In acute intermittent porphyria
 - e. In Cushing's syndrome
25. Ultrasound pattern in Graves Basedow disease:
- a. The thyroid volume can be increased, normal or decreased
 - b. Usually no nodules are detected
 - c. The thyroid parenchyma is hyperechoic
 - d. There is increased diffuse vascularization
 - e. „Thyroid inferno" is also encountered in Hashimoto thyroiditis
26. Regarding antithyroid drugs in Graves Basedow disease:
- a. Methimazole induces agranulocytosis in 5% of
 - b. Propylthiouracil can induce hepatotoxicity
 - c. Can be administered indefinitely in Graves^[SEP] Basedow disease
 - d. Propylthiouracil is contraindicated in pregnancy^[SEP]
 - e. Induce remission of the disease after 1,5-2 years in 50-80% of cases
27. Graves ophtalmopathy:
- a. Female gender is a risk factor
 - b. Iatrogenic hypothyroidism is a risk factor
 - c. It is present in all cases of graves Basedow disease
 - d. Occurs most often before the diaonosis of Graves Basedow hyperthyroidism
 - e. It is concomitant with the diagnosis Basedow disease in 50% of cases
28. Regarding treatment of toxic adenoma
- a. Radioiodine therapy is indicated in young patients
 - b. Radioiodine therapy is indicated for small nodules
 - c. Radioiodine therapy is given when there is no preplanned pregnancy
 - d. Lobectomy is indicated for big, compressive nodules
 - e. After lobectomy, hypothyroidism ensures
29. Regarding hypothyroidism of the pregnant woman
- a. Supplemental therapy of the mother will also sustain the fetus
 - b. T4 and T3 preparations are allowed as treatment
 - c. Breastfeeding is not allowed
 - d. TSH target in the first trimester is < 3mUI/L
 - e. TSH target in the second and third trimester is < 3mUI/L
30. Full thyroid replacement dose as a function of age
- a. 1-2 µg/kg/day levo-thyroxine in children aged 6-10 years
 - b. 10-15 µg/kg/day levo-thyroxine in newborns aged 0-6 months
 - c. 5-6 µg/kg/day levo-thyroxine in children aged 1-5 years
 - d. 6-8 µg/kg/day levo-thyroxine in children aged 11-20 years
 - e. In thyroid cancer patients 2,2 µg/kg/day of levo-thyroxine
31. During the aggression phase of subacute thyroiditis

- a. The ESR can be > 100 mm/h
 - b. Thyroglobulin level is increased, reflecting increased thyroid activity
 - c. Anti-TPO antibodies are increased
 - d. Radioactive iodine uptake is increased
 - e. Ultrasound demonstrates a patchy aspect of the thyroid
32. Riedel's thyroiditis
- a. It is a rare fibrotic condition that involves the thyroid gland
 - b. Patients are more likely to be adult middle aged men
 - c. It is a painful woody goiter
 - d. Obstructive symptoms are frequent
 - e. Tamoxifen is a SERM used as treatment for Riedel's thyroiditis
33. Obstructive symptoms of nodular goiter
- a. Dysphagia
 - b. Inspiratory dyspnea
 - c. Claude Bernard Horner's syndrome: mydriasis, ptosis and enophthalmos
 - d. Dysphonia
 - e. Pemberton sign; sudden weakness in the muscles on one side of the face
34. Follow up of nodules after FNAB
- a. In case of benign cytology, with highly suspicious ultrasound features – US follow up
 - b. In case of intermediate cytology (Bethesda III, IV) in high risk US nodules – surgical indication
 - c. In case of malignant cytology (Bethesda V) – repeat FNAB in 12 months
 - d. In case of malignant cytology (Bethesda VI) – absolute total thyroidectomy
 - e. In case of benign cytology, with low suspicious US features – repeat FNAB in 24 months
35. High risk features of thyroid nodules in ultrasound are
- a. Isoechogenicity of the nodule
 - b. Ill defined margins
 - c. A present halo sign
 - d. Taller than wide nodules
 - e. Marked increase in elasticity
36. Primary osteoporosis type II
- a. Affects women predominantly
 - b. Occurs after 70 years of age
 - c. Explained by the fact that bone formation gradually declines with age
 - d. Affects predominantly trabecular bone
 - e. There is an increased risk of femoral neck fractures
37. Differential diagnosis of hypercalcemia in primary hyperparathyroidism is made with
- a. Hypothyroidism
 - b. Paraneoplastic syndrome
 - c. Addison's disease
 - d. Treatment with loop diuretics
 - e. Treatment with bisphosphonates
38. Progesterone
- a. Decreases body temperature
 - b. Stimulates myometrial contractility

- c. Stimulates the release of mature oocytes
 - d. Facilitates implantation
 - e. Stimulates breast cell proliferation
39. Regarding cinacalcet
- a. It binds to the CaSR and decreases its sensitivity to extracellular calcium
 - b. Decreases PTH values by around 50%
 - c. Significantly decreases serum phosphate levels
 - d. It is approved for postsurgical relapse of primary hyperparathyroidism
 - e. It is approved for parathyroid carcinoma
40. Bone forming medication for osteoporosis
- a. Denosumab
 - b. Strontium ranelate
 - c. Calcitonin
 - d. Teriparatide (Forsteo)
 - e. Bisphosphonates
41. Complications of corticotherapy
- a. Acute pancreatitis
 - b. Potassium retention
 - c. Depression
 - d. Growth retardation in children
 - e. Hypocoagulability
42. Screening tests in Cushing's syndrome
- a. Urinary free cortisol evaluates total cortisol
 - b. Nocturnal salivary cortisol demonstrates the alteration of circadian rhythm
 - c. The dexamethasone suppression test demonstrates altered feedback of the HPA axis
 - d. Urinary free cortisol demonstrates hypersecretion of cortisol
 - e. Nocturnal salivary cortisol is influenced by the variation of CBG concentration
43. Regarding the medical treatment of Cushing's disease
- a. Pasireotide has a higher affinity to somatostatin receptor type 2 than type 5, similar to the first generation somatostatin receptor analogues
 - b. Ketoconazole has a risk for hepatotoxicity
 - c. Metyrapone inhibits 17-alpha-hydroxylase
 - d. Osilodrostat inhibits both 11-beta-hydroxylase and aldosterone synthetase
 - e. Mitotane is adrenolytic and used as treatment for adrenocortical carcinoma
44. Addison's disease is characterized by
- a. Hyperpigmentation
 - b. Metabolic alkalosis
 - c. Polycythemia
 - d. Weakness, fatigue, weight loss
 - e. Gastro-intestinal disturbances: nausea, vomiting, diarrhea
45. Regarding pseudohypoparathyroidism
- a. It is a rare inherited disorder characterized by target organ resistance to PTH
 - b. Elevated levels of PTH are present in serum
 - c. The electrocardiogram detects shortened QT interval

- d. A positive Chvostek sign is when inflation of a sphygmomanometer to higher than systolic blood pressure and maintain that pressure for 3 minutes, leads to transient ischemia and carpo-pedal spasm
 - e. There is hypocalcemia and hyperphosphatemia
46. Drugs that can trigger paroxysms in pheochromocytoma
- a. MAO inhibitors
 - b. Metyrosine
 - c. Glucagon
 - d. Methyldopa
 - e. Labetolol
47. McCune Albright syndrome is characterized by
- a. Fibrous dysplasia of the bones
 - b. Delayed puberty
 - c. Café au lait spots
 - d. Addisons disease
 - e. GH secreting pituitary adenomas
48. Primary hyperaldosteronism
- a. Aldosterone producing adenomas in 65% of cases
 - b. Adrenal carcinoma in 15% of cases
 - c. Type I familial hyperaldosteronism (glucocorticoid remediable) in 1% of cases
 - d. Bilateral hyperplasia in 65% of cases
 - e. Type II familial hyperaldosteronism (glucocorticoid unremediable) in 2% of cases
49. Regarding hypertension in hypothyroidism
- a. It is a systolic hypertension
 - b. It is associated with increased sensitivity to circulating catecholamines
 - c. It is associated with tachycardia
 - d. It is associated with extracellular volume expansion
 - e. It is associated with increased peripheral resistance
50. Dihydrotestosterone effects in men
- a. Induces masculinization of external genitalia in fetal life
 - b. Induces bone growth
 - c. Has anabolic effects on muscle
 - d. Induces testes descent in fetal life
 - e. Stimulates libido

Endo 2023
Re-session

19. GnRH is produced in the:

- A. preoptic septum
- B. anterior-medial hypothalamus
- C. arcuate nuclei
- D. paraventricular nuclei
- E. anterior pituitary

20. Adrenal supplemental therapy should be increased by 5-10 mg in case of:

- A. major surgery
- B. delivery
- C. dehydration
- D. emotional stress
- E. pregnancy

21. The differential diagnosis in a female patient with hyperprolactinemia is:

- A. pregnancy
- B. Hypothyroidism
- C. polychistic ovarian syndrome
- D. hyperthyroidism
- E. Cushing disease

22. Surgery is recommended in asymptomatic hyperparathyroidism if:

- A. GFR < 60 ml/min
- B. Age < 50 years
- C. Age > 50 years
- D. Ca > 2 mg/dl upper normal value
- E. Ca > 1 mg/dl upper normal value

23. Thyroid binding globulin is decreased by:

- A. androgens
- B. nephrotic syndrome
- C. malnutrition
- D. pregnancy
- E. acute intermittent porphyria

24. The major clinical signs in Graves disease are:

- A. insomnia
- B. Tachycardia
- C. exophthalmia
- D. Tremor
- E. weight loss

25. Risk factors for severe ophthalmopathy are:

- a. female gender
- B. male gender

- C. multiple relapses
- D. iatrogenic by pothyroidism
- E. alcoholism

26. The indications for radioiodine treatment in Graves" disease patients are:

- A. nodular goitre
 - B. old age
 - C. severe ophthalmopathy
 - D. young age
 - E. no response to the first line of treatment
- 27 ??

28. Causative factors for acute thyroiditis are:

- A. sinus fistulas
- B. persistent thyroglossal comants
- C. thyroid surgery wound infection
- D. recent viral infections *SUBACUTE*
- E. often, after FNAB *RARE*

29. The recovery phase of subacute thyroiditis as defined by:

- A. inflammation
- B. High thyroglobulin
- C. hypothyssisto
- D. anemia
- E increased RAIU

30. Thyroid nodules with intermediate fisk on ultrasound have:

- A. ischemic appearance
- B. partial halo sign
- C. egg shell calcifications
- D. slightly increased elasticity
- E. ill defined limits *HIGH*

31. Surgery is recommended in nodular gutter with:

- A. compression
- B. substernal entersion
- C. Hypothyroidism
- D. autonomous function
- E. malignancy suspicion

32. Radioactive iodine treatment is recommended in patients with thyroid cancer and:

- A. disstant metastasis *USEFUL ONLY IN TOTAL THYREIDECTOMY*
- B. partial thyroidectomy
- C. Extrathyroidal extension
- D. Lymph node invasion
- E. vascular invasion

33. Treatment for acute hypercalcemic crisis include:

- A. Furosemide
- B. Biphosphomates
- C. Calcitonin
- D. steroids
- E. thiazidic diuretics

34. Increased levels of 1,25-OH-D are associated the following pathologies:

- A. primary hyperparathyroidism
- B. sarcoidosis
- C. inflammatory bowel disease
- B. hyperthyroidism
- E. milk-alkali syndrome

35. Cardiac effects of hypercalcemia include:

- A. tachycardia
- B. diastolic hypertension
- C. orthostatic hypotension
- D. long QT
- E. left ventricular hypertrophy

36. FGF 23 :

- A. is produced by osteoclasts and osteoblasts
- B. regulates calcium homeostasis in the body
- C. decreases the activity of 25-OH-hydroxylase
- D. decreases renal phosphate resorbtion
- E. is inhibited by hyperphosphatemia

37. In patients with iatrogenic Cushing Syndrome:

- A. ACTH levels are low
- B. ACTH levels are high
- C. cortisol levels are low
- D. cortisol levels are high
- E. CT reveals adrenal hyperplasia due to prolonged corticotherapy

38. A patient developed permanent hypoparathyroidism after total thyroidectomy. What are the appropriate follow-up investigations and treatment:

- A. follow-up with total calcemia and calciuria
- B. follow-up with total calcemia and calcitonin
- C. treatment with calcium and vitamin D3 supplements *DECREASE INTAKE*
- D. treatment with calcium and calcitriol *DECREASE ACTIVATION*
- E. treatment with calcium and phosphate supplements

39. The metabolic effects of cortisol includes:

- A. decreased nitrogen excretion
- B. increased gluconeogenesis
- C. increased fat mobilization
- D. anti-insulin action
- E. increased antiinflammatory cytokine production *DECREASE PROINFLAMMATORY*

40. The drugs that trigger paroxysm in pheochromocytoma are:

- A. nicotine
- B. omeprazole
- C. MAO inhibitors
- D. cocaine
- E. opiates

41. Possible complications of iatrogenic Cushing syndrome include:

- A. appendicitis
- B. pancreatitis
- C. glaucoma
- D. growth impairment in children
- E. intracranial hypotension

42. The non-classic effects of aldosterone are:

- A. sodium reabsorption
- B. expression of collagen genes
- C. microangiopathy
- D. tubular potassium secretion
- E. Inactivation of tissular growth factors and inflammation **ACTIVATION**

43. Symptoms of hypokalemia are:

- A. muscle weakness
- B. palpitations
- C. abdominal distension
- D. dyspnea
- E. permanent paralysis **TEMPORARY**

44. Special categories of population at risk of osteoporosis are:

- A. Elderly Patients
- B. patients with sarcopenia
- C. long-standing hyperthyroid patients
- D. postmenopausal women
- E. short-term glucocorticoid therapy

45. The treatment options for medullary thyroid carcinoma include:

- A. suppressive levothyroxine therapy
- B. radioactive iodine therapy
- C. total thyroidectomy
- D. chemotherapy with tyrosine kinase inhibitors
- E. external radiotherapy

46. Contraindications for sex hormone replacement therapy in women include:

- A. breast cancer
- B. hepatic insufficiency
- C. chronic porphyria
- D. mild hypertension

E. rheumatoid arthritis

47. Adrenal dependent causes of endocrine hypertension include:

- A. Cushing disease
- B. Deoxycorticosteron producing tumor
- C. 11-beta-hydroxylase deficiency
- D. 17-alpha-hydroxylase deficiency
- E. 21-alpha-hydroxylase deficiency

48. Active screening for MEN2 syndrome includes tests:

- A. pituitary MRI
- B. calcitonin
- C. free plasma metanephrines and normetanephrines
- D. PTH
- E. serum catecholamines

49. Inherited hypopituitarism may be caused by:

- A. GHRH receptor mutations
- B. Kallmann Syndrome
- C. pituitary aplasia
- D. Sheehan syndrome
- E. autoimmune infiltration of the pituitary gland

50. Neck CT or MRI is recommended in patients w goiter for the evaluation of:

- A. compressive symptoms
- B. regional lymph node
- C. intrathyroidal nodules
- D. malignancy suspicion
- E. invasive primary thyroid cancer extends

Endo 2023 re-re

Test 3B

1. * Prolactin hypersecretion is confirmed by the lack of response to stimulation with:

- A. Insulin
- B. GhRH
- C. TRH
- D. TSH
- E. Dopamine

2. * Tetany occurs when serum calcium levels drop below:

- A. 7 mg/dl
- B. 7,5 mg/dl
- C. 8 mg/dl
- D. 8,5 mg/dl
- E. 6 mg/dl

3. * Lack of response to antiosteoporotic treatment is characterized by:

- A. bone loss > 10%/year
- B. bone loss > 5 %/year
- C. fragility fracture before treatment
- D. adverse effects of the treatment
- E. no change on DXA in bone mineral density after one year of treatment

4. * Pseudo-diabetes insipidus is caused by:

- A. altered vasopressin production
- B. increased vasopressin metabolism
- C. altered vasopressin reception
- D. decreased vasopressin needs
- E. decreased vasopressin metabolism

5. * CRH is secreted in the:

- A. arcuate nucleus
- B. anterior periventricular nucleus
- C. preoptic septum anterior pituitary
- D. anterior pituitary
- E. medial paraventricular nucleus

6. * The most frequent type of pituitary adenoma is:

- A. GH-secreting adenoma
- B. ACTH-secreting adenoma
- C. prolactinoma
- D. tireotropinoma
- E. craniopharyngioma

7. * WHO recommendations for daily iodine intake in adults are:

- A. 200 mcg
- B. 50 mcg
- C. < 100 mcg
- D. 500 mcg
- E. 150 mcg

8. * The major clinical signs in Graves' disease

- A. Tachycardia
- B. weight loss
- C. tremor
- D. insomnia
- E. excessive sweating

9. * The third line of treatment in GH-secreting adenomas is represented by:

- A. radiotherapy
- B. Somatostatin analogues
- C. surgery
- D. Pegvisomant
- E. Cabergoline

10. * The most common cause of permanent hypoparathyroidism is:

- A. iatrogenic- parathyroid excision or ischemia after thyroidectomy
- B. autoimmune
- C. dysgenesie
- D. infiltration in sarcoidosis
- E. idiopathic

11. The chronic adverse effects of pituitary radiotherapy are:

- A. optic nerve damage
- B. IQ alterations in adults
- C. stroke risk
- D. alopecia e.
- E. cranial nerve alteration

12. Cabergoline:

- A. has a long half life b.
- B. may cause cardiac valvular lesions
- C. could increase the incidence of Parkinson's disease
- D. stimulates DOPA receptors agonist in the pituitary
- E. has a worse tolerance than bromocriptine

13. Female patient, 28 years old, is admitted in the ER with syncope, abdominal pain, weight loss, vomiting and amenorrhea. The symptoms started after the delivery of her second child, when she developed severe postpartum hemorrhage. She was not able to breastfeed the newborn. What is the most probable diagnosis:

- A. Sheehan syndrome
- B. postpartum pituitary necrosis
- C. Addison' disease

- D. Myxedema
- E. postpartum depression

14. Thyroid scintigraphy is indicated in:

- A. hyperthyroidism
- B. ectopic goitre
- C. post thyroidectomy evaluation
- D. hypothyroidism
- E. malignancy suspicion

15. Antithyroid medication may have the following side effects:

- A. hepatotoxicity
- B. skin rash
- C. vasculitis
- D. cardiotoxicity
- E. anemia

16. Drugs that may trigger paronychia in pheochromocytoma are:

- A. alpha-blockers
- B. glucagon
- C. methyldopa
- D. insulin
- E. nicotine

17. IGF-1 mediated effects of GH are:

- A. bone formation
- B. protein synthesis
- C. lipolysis
- D. gluconeogenesis
- E. cartilage growth

18. The differential diagnosis of gigantism includes the following pathologies:

- A. Marfan syndrome
- B. Klinefelter syndrome
- C. precocious puberty
- D. late-onset puberty
- E. genetic decreased height

19. Causes of GH/IGF1 hypersecretion, other than a GH-secreting adenoma, are:

- A. anxiety
- B. starvation
- C. obesity
- D. CKD
- E. acute kidney disease

20. The second phase of acute pituitary insufficiency includes:

- A. ophthalmoplegia
- B. hypotension

C. mental deterioration

D. meningismus

E. severe headache

21. Contraindications for androgen supplemental therapy are:

A. prostate cancer

B. mild hepatic disease

C. well-controlled arterial hypertension

D. sleep apnoea

E. breast cancer

22. Drugs that prolong the action/release of pressin are:

A. clofibrate

B. indomethacin

C. carbamazepine

D. desmopressin

E. Ketoconazol

23. Avoid stimulation by ectopic betaHCG, when occurs in:

A. Hydatidiform mole

B. Chorioblastic tumors

C. Hyperemesis gravidarum

D. ian adenocarcinoma

E.. Struma ovari

24. Which of the following clinical risk factors suggestive for malignancy in a patient with goiter:

A. Vocal cord paralysis

B. Firmness on palpation

C. nodules > 2 cm in diameter

D. Symptoms of hyperthyroidism

E. suspicious ultrasound features

25. On thyroid d ultrasound, high risk thyroid nodules have:

A. clear margins

B. significantly increased elasticity

C. «taller then wide" shape

D. intranodular calcifications

E. homogenous straelent

26. Proopiomelanocortin (POMC) posttranslational dissociation results int

A. ACTH

B. alpha- MSH

C. LPH

D. CRH

E. Cortisol

27. The mass effect of pituitary tumors may trigger:

- A. bitemporal hemianopsia from down to up
- B. loss of blue perception
- C. scotoma
- D. appetite disturbances
- E. loss of red perception

28. Active screening for hyperprolactinemia is recommended in the following cases:

- A. enlarged sella turcica
- B. polimenorrhea
- C. suspicion of hypothalamic adenoma
- D. male hypogonadism
- E. male infertility

29. Prolactin can be increased in:

- A. thoracic lesions
- B. medullary lesions
- C. hypothyroidism
- D. metoclopramide therapy
- E. pseudohypoparathyroidism

30. Myxedematous cretinism can be caused by:

- a. transplacental passage of stimulatory TSH receptor antibodies
- B. inborn dyshormonogenesis
- C. exposure to antithyroid medication
- D. iodine deficiency
- E. thyroid agenesis

31. Treatment in subclinical hypothyroidism administered in case of:

- A. tachycardia
- B. old patients
- C. young patients
- D. dyslipidemia
- E. anemia